most will die at a young age, sometimes within the first week—to help you decide what to do. In some cases these disorders are associated with abnormalities detected during the dating scan (see First ultrasound scan), and the presence of these abnormalities combined with a positive test may confirm the diagnosis. Nevertheless, many women opt to have a diagnostic test before deciding whether or not to terminate their pregnancy.

More screening

Another option is to have a special ultrasound scan to look for Down syndrome markers between 17 and 22 weeks. However, a second-trimester ultrasound is not such an accurate test for Down syndrome as other tests, so only consider this path only if you are strongly against having a diagnostic test such as amniocentesis and feel that you don't need a definite diagnosis.

PAPP-A

PAPP-A (Pregnancy Associated Plasma Protein-A) is produced by the placenta and can be used to screen for growth problems and preeclampsia. If a blood test shows the PAPP-A level is low (this occurs in about 5 percent of pregnancies), there is an increased risk of preeclampsia or of having a low birthweight baby. You will be offered a specialized scan at 20 weeks to look at the blood flow to the placenta and extra growth scans later on in pregnancy. Low-dose aspirin, which can reduce the risk of preeclampsia, will also be suggested as long as you do not have an allergy to aspirin. If you smoke, this is definitely the time to quit.

NUCHAL TRANSLUCENCY SCREENING

An ultrasound scan, known as the nuchal translucency screening, is done between 11 and 14 weeks of pregnancy and is used to help assess your baby's risk of Down syndrome. During this test, the sonographer measures the depth of fluid under the skin at the back of the baby's neck, known as the nuchal fold. If this measurement is high, it indicates that excess fluid is present, which means that the baby has a higher risk of Down syndrome. The measurement is then combined with the results of blood tests and the average risk based on your age to give your baby's individual risk for Down syndrome. If your risk is higher than 1 in 250, you will be counseled and offered a diagnostic test, such as amniocentesis or chorionic villus sampling, to give a definitive result.